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TOPIC: COMPLICATIONS DURING PREGNANCY AND IT'S MANAGEMENT

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Complications During Pregnancy



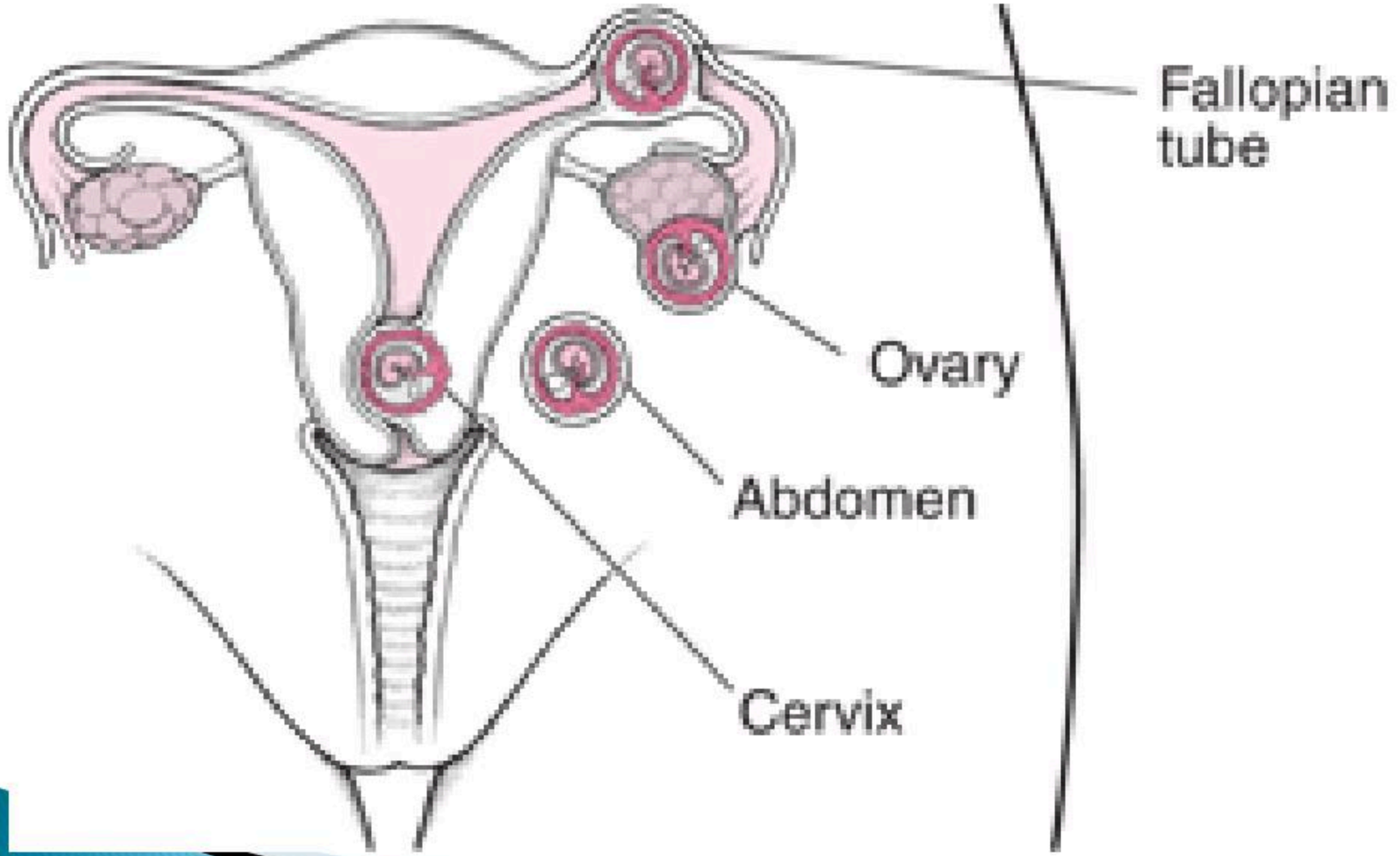
Ectopic Pregnancy

- Normally, an egg is fertilized in the fallopian tube and becomes implanted in the uterus. However, if the tube is narrowed or blocked, the egg may move slowly or become stuck. The fertilized egg may never reach the uterus, resulting in an ectopic pregnancy. Ectopic pregnancies usually develop in one of the fallopian tubes (as a tubal pregnancy) but may develop in other locations. A fetus in an ectopic pregnancy cannot survive.
- One of 100 to 200 pregnancies is an ectopic pregnancy.
- Symptoms include unexpected vaginal bleeding and cramping. If the fallopian tube ruptures (typically after about 6 to 8 weeks), a woman usually feels severe pain in the lower abdomen and may faint. If the tube ruptures later (after about 12 to 16 weeks), the risk of death for the woman is increased, because the fetus and placenta are larger and more blood is lost.

Preeclampsia

- About 5% of pregnant women develop preeclampsia (toxemia of pregnancy). In this complication, an increase in blood pressure is accompanied by protein in the urine (proteinuria).
- Preeclampsia usually develops between the 20th week of pregnancy and the end of the first week after delivery.
- The cause of preeclampsia is unknown. But it is more common among women who are pregnant for the first time, who are carrying two or more fetuses, who have had preeclampsia in a previous pregnancy, who already have high blood pressure or a blood vessel disorder, or who have sickle cell disease. It is also more common among girls aged 15 and younger and among women aged 35 and older.

Ectopic Pregnancy



Gestational Diabetes

- About 1 to 3% of pregnant women develop diabetes during pregnancy. This disorder is called gestational diabetes. Unrecognized and untreated, gestational diabetes can increase the risk of health problems for pregnant women and the fetus and the risk of death for the fetus.
- Gestational diabetes is more common among obese women and among certain ethnic groups, particularly Native Americans, Pacific Islanders, and women of Mexican, Indian, and Asian descent.

Gestational Diabetes

- Most women with gestational diabetes develop it because they cannot produce enough insulin. Some women may have had diabetes before becoming pregnant, but it was not recognized until they became pregnant.
- A blood test is used to measure the blood sugar level. Women who have gestational diabetes are usually taught to measure their blood sugar levels with a home blood sugar monitoring device.
- Treatment consists of eliminating high-sugar foods from the diet, eating to avoid excess weight gain during the pregnancy, and, if the blood sugar level is high, taking insulin.
- After delivery, gestational diabetes usually disappears. However, many women who have gestational diabetes develop type 2 diabetes as they become older.

Rh Incompatibility

- Rh incompatibility occurs when a pregnant woman has Rh-negative blood and the fetus has Rh-positive blood, inherited from a father who has Rh-positive blood. In about 13% of marriages in the United States, the man has Rh-positive blood and the woman has Rh-negative blood.
- Blood is Rh-positive if red blood cells have the Rh factor and Rh-negative if they do not. Problems can occur if the fetus's Rh-positive blood enters the woman's bloodstream. The woman's immune system may recognize the fetus's red blood cells as foreign and produce antibodies, called Rh antibodies, to destroy the fetus's red blood cells.

Rh Incompatibility

- As a precaution, women who have Rh-negative blood are given an injection of Rh antibodies at 28 weeks of pregnancy and within 72 hours after delivery of a baby who has Rh-positive blood, even after a miscarriage or an abortion.

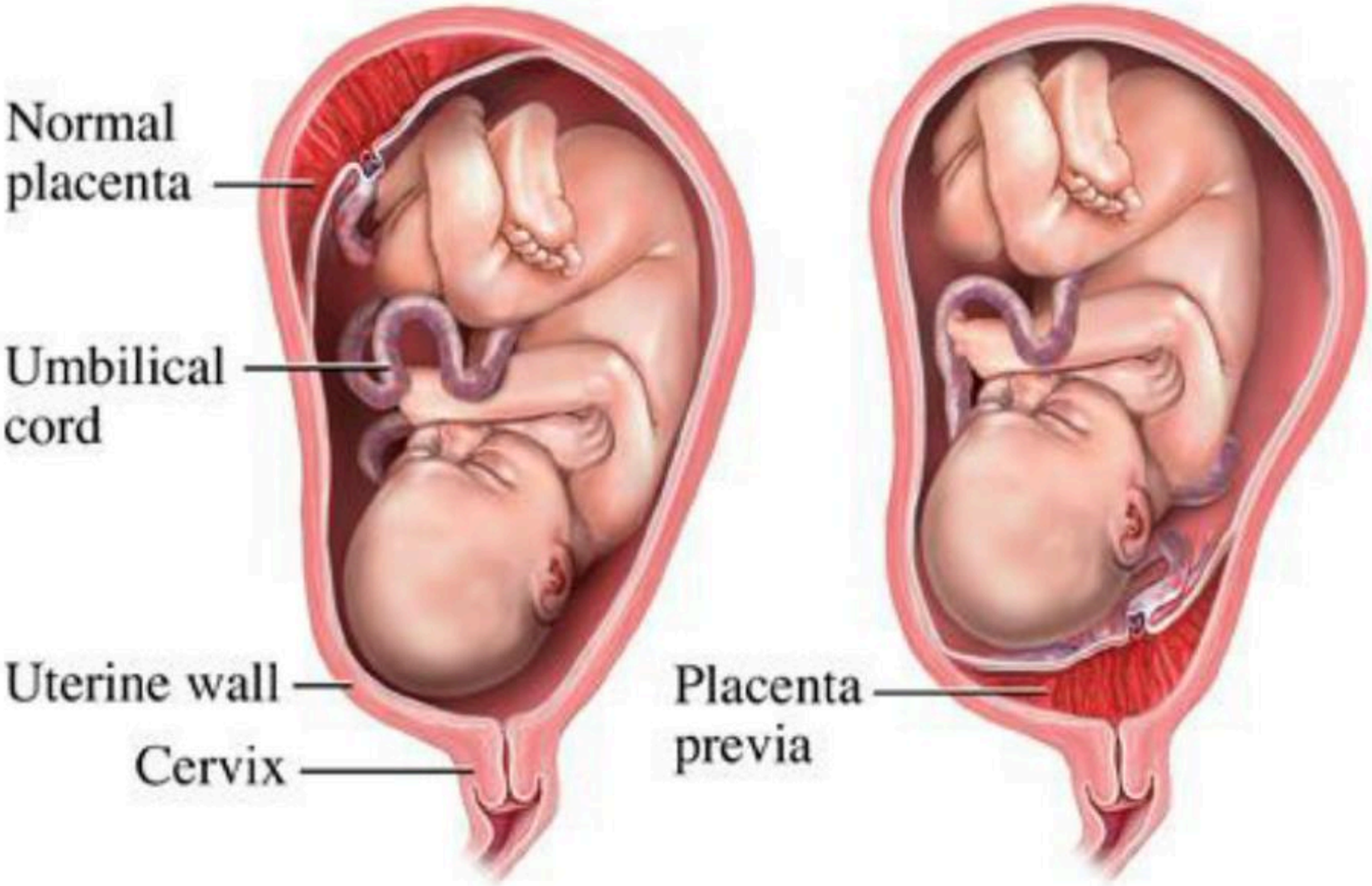
Placenta Previa

- Placenta previa is implantation of the placenta over or near the cervix, in the lower rather than the upper part of the uterus. The placenta may completely or partially cover the opening of the cervix. Placenta previa occurs in 1 of 200 deliveries, usually in women who have had more than one pregnancy.
- Placenta previa can cause painless bleeding from the vagina that suddenly begins late in pregnancy. The blood may be bright red. Bleeding may become profuse, endangering the life of the woman and the fetus.

Problems With Amniotic Fluid

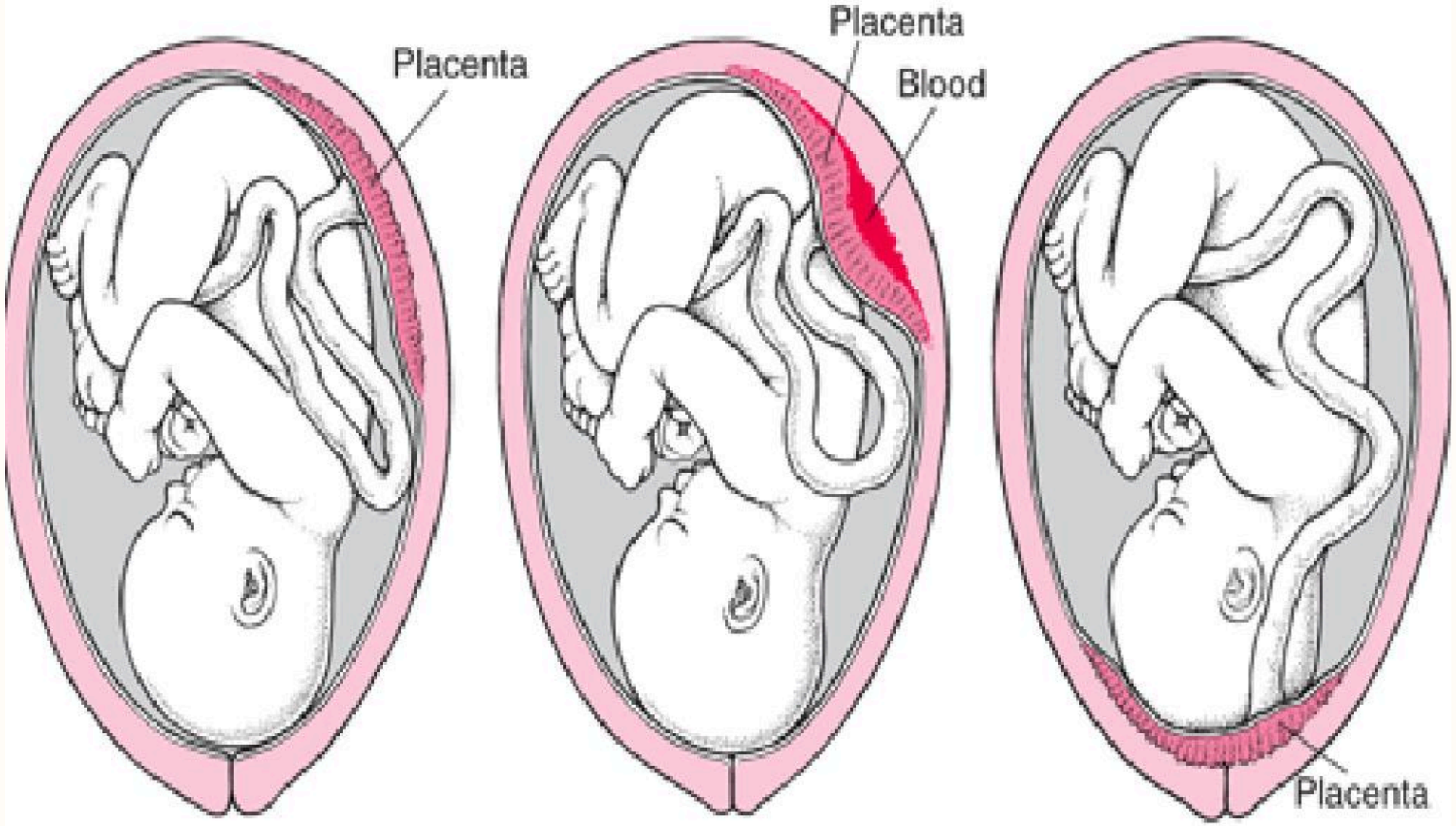
- Too much amniotic fluid in the membranes containing the fetus (amniotic sac) stretches the uterus and puts pressure on the diaphragm of pregnant women. This complication can lead to severe breathing problems for the women or to preterm labor.
- Too little amniotic fluid (oligohydramnios) can also cause problems. If the amount of fluid is greatly reduced, the fetus's lungs may be immature and the fetus may be compressed, resulting in deformities; this combination of conditions is called Potter's syndrome.
- Too little amniotic fluid tends to develop when the fetus has birth defects in the urinary tract, has not grown as much as expected, or dies.

Placenta Previa



Placental Abruption

- Placental abruption is the premature detachment of a normally positioned placenta from the wall of the uterus. The placenta may detach incompletely (sometimes just 10 to 20%) or completely. The cause is unknown. Detachment of the placenta occurs in 0.4 to 3.5% of all deliveries. This complication is more common among women who have high blood pressure (including preeclampsia) and among women who use cocaine.
- When the placenta detaches, the supply of oxygen and nutrients to the fetus may be reduced.


Placenta
Placenta
Blood
Placenta
Normal Placenta
Placental Abruption
Placenta Previa

THANK YOU